MAPLE RIDGE PRIMARY CARE - OFFICE VISIT

Date of service: 08/24/2026 | Provider: Renee Ashworth, MD

Chief complaint

Low back pain going down the right leg.

History of present illness

43-year-old warehouse supervisor. Low back pain began about 08/10/2026 after he lifted a pallet at work. The pain travels down the back of the right thigh to the calf. It is 7 out of 10, worse with sitting and bending. He took a few doses of over-the-counter ibuprofen with little effect. He denies bowel or bladder changes, saddle numbness, fever, and weight loss.

Past medical history

None significant. No prior back surgery. No history of cancer. No known drug allergies.

Examination

BP 126/80, HR 74, BMI 28. Tender over the right lower lumbar paraspinal muscles. Straight leg raise positive on the right at 40 degrees. Strength 5/5 in both legs. Reflexes symmetric. Sensation intact.

Assessment

Acute lumbar radiculopathy, right (ICD-10 M54.16).

Plan

- Start naproxen 500 mg twice daily with food.
- Referral to physical therapy.
- Work restriction: no lifting over 20 pounds.
- Return in 6 weeks, sooner for leg weakness or bladder symptoms.

Electronically signed: Renee Ashworth, MD, 08/24/2026 09:15

MAPLE RIDGE IMAGING - RADIOLOGY REPORT

Examination: lumbar spine X-ray, 2 views | Date: 08/24/2026 | Ordered by: Renee Ashworth, MD

Indication

Low back pain with right leg symptoms after lifting.

Findings

Five lumbar-type vertebrae with normal alignment. Vertebral body heights are preserved. Mild disc space narrowing at L5-S1. No fracture or destructive bone lesion. Sacroiliac joints are unremarkable.

Impression

Mild degenerative change at L5-S1. No acute abnormality.

Electronically signed: Hannah Voss, MD, Radiology, 08/24/2026 15:31

STRIDE PHYSICAL THERAPY - INITIAL EVALUATION

Date: 09/01/2026 | Therapist: Carlos Mendes, PT | Referred by: R. Ashworth, MD

Subjective

Right-sided low back pain with leg symptoms for 3 weeks. Pain 6 to 7 out of 10. Sitting tolerance 20 minutes. Works on modified duty.

Objective

Lumbar flexion limited to 50 percent with leg pain. Extension reduces leg symptoms. Straight leg raise positive on the right. Oswestry Disability Index 44 percent.

Assessment

Findings consistent with a right lumbar radiculopathy that responds to extension. Good candidate for therapy.

Plan of care

Physical therapy 2 visits per week for 6 weeks (12 visits). Directional preference exercises, nerve mobility work, core stabilization, and body mechanics training.

Home exercise program handout issued (prone press-ups and nerve glides), to be done daily.

Electronically signed: Carlos Mendes, PT, 09/01/2026 17:02

STRIDE PHYSICAL THERAPY - DISCHARGE NOTE

Date: 09/29/2026 | Therapist: Carlos Mendes, PT

Attendance

The patient attended 8 of 12 planned visits between 09/01/2026 and 09/29/2026, a period of 4 weeks. He cancelled the remaining visits because of a change in his work schedule and was discharged from physical therapy on 09/29/2026.

Attendance log

- Attended: 09/01, 09/03, 09/08, 09/10, 09/15, 09/17, 09/22, 09/29.
- Cancelled by patient: 09/24, 10/01, 10/06, 10/08.

Status at discharge

Pain 6 out of 10. Oswestry Disability Index 40 percent. Sitting tolerance 30 minutes. Leg symptoms unchanged. Minimal overall change.

Recommendation

Continue the home exercises independently. Return to the referring physician.

Electronically signed: Carlos Mendes, PT, 09/29/2026 16:44

MAPLE RIDGE PRIMARY CARE - OFFICE VISIT

Date of service: 10/06/2026 | Provider: Renee Ashworth, MD

Interval history

Re-evaluation after conservative therapy: right-sided low back and leg pain persists at 6 out of 10 with no meaningful improvement. Symptoms have now been present for about 8 weeks, with onset on 08/10/2026. He has taken naproxen 500 mg twice daily for six weeks with minimal relief. He says he has done his home exercises most days since leaving physical therapy.

Examination

Straight leg raise positive on the right at 35 degrees. Strength is 5/5 in both legs. No saddle anesthesia. He denies bowel or bladder dysfunction, fever, and unexplained weight loss, and has no history of cancer. Sensation is mildly reduced over the outer right foot. Ankle reflex slightly reduced on the right.

Assessment

Right lumbar radiculopathy, persistent after medication and physical therapy.

Plan

Order MRI of the lumbar spine without contrast to look for a disc herniation with nerve root compression. Consider referral for an epidural steroid injection depending on the result. Continue naproxen and work restrictions.

Electronically signed: Renee Ashworth, MD, 10/06/2026 10:52